currency had to be deposited in the National Bank and exchanged for pesos.

Experiments with incentives that fostered inequality only went so far. The government's commitment to equality continued to reign in other sectors, particularly in the area of health. Institutionalization in health care meant the creation of regional structures following the Soviet model. But, unlike the Soviets, the Cubans continued to invest heavily in their health system, and to emphasize primary care and community health initiatives. Cuba's innovative and comprehensive system of polyclinics not only put accessible clinics everywhere in the country, it "projects the polyclinics services into the neighborhood. Not only do the physician–nurse teams attend patients in the polyclinic, but they also visit patients in their everyday environment: the home, school, day-care center, and workplace."[25] In 1984 the country implemented a new "family doctor" program – a drive to train thousands of primary care doctors and place a doctor in every neighborhood of the country. The idea was to integrate primary medical care into the community. The number of health care workers tripled between 1970 and 2005, while the number of doctors rose from "one doctor for every 1,393 people in 1970 to one doctor for every 159 people in 2005."[26]

Human capital was also maximized through voluntarism and the mass organizations. Workers were asked to volunteer for "micro-brigades" to construct housing. CDRs and the FMC (Federación de Mujeres Cubanas, or Federation of Cuban Women) played a key role in the health system. "Together, they can reach into all neighborhoods of Cuba, since CDRs exist in every block and every little town," an education official explained to Jonathan Kozol in 1976. "If an elderly person, for example, is not seen out on the street for several days, it is the obligation of the CDR to go upstairs into that person's home and to find out if he or she is incapacitated somehow. The CDR and the FMC work in close cooperation with the Ministry of Health. Proper prenatal care depends upon the use of polyclinics, but polyclinics can be of no use to anyone if women do not recognize the need to keep appointments … If a woman fails to show up for appointments, someone from the CDR goes to her